



Fiona Stanley Fremantle Hospitals Group

Procedure

Did not attend: outpatient appointments

Reference #: FSFH-MENH-PRO-0001

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service Alcohol and Other Drug Service	Medical, Nursing, Allied Health

1. Introduction

This procedure guides staff in appropriately following up consumers who did not attend their scheduled outpatient appointment or group program. It is imperative these consumers are risk assessed early to ensure timely support is provided if required.

2. Terminology

Did not attend	When a consumer does not attend their scheduled/agreed appointment with any clinician within the mental health service setting without notifying the service prior to appointment time.
Risk Assessment	Information gathering to identify relevant risk factors to the consumer and others from a longitudinal, cross-sectional, and unaddressed risk approach and developing a plan to mitigate risk. Documented on the Risk Assessment and Management Plan (RAMP).
PSOLIS/webPSOLIS	Psychiatric Online Service Information System (PSOLIS) is the patient database used by WA public mental health services. webPSOLIS contains Statewide Standardised Clinical Documentation.
Community mental health assessment	An assessment performed by a mental health clinician to determine: - the presence of mental health symptoms or physical illness - if there is a significant risk to the person or another person if treatment is not provided,

	• whether the person is well enough to be able to make an informed decision about their own treatment, • what treatment and supports are most appropriate through shared decision making with the consumer and their personal support person
Personal support persons	Under the MHA 2014, is a broad term referring to any of the following: carer, family member, Elder or traditional healer, Nominated Person ² .

3. Procedure

3.1. Involvement of the personal support person

- Actively involving the consumer and their personal support person in the assessment and treatment process can facilitate improved consumer outcomes.
- The clinician will support the involvement of the consumers nominated personal support person through co-designing with the consumer a plan for:
 - when information is shared for example: if the patient does not attend an appointment and is unable to be contacted the service will contact the nominated personal support person.
 - what information does and does not need to be shared
 - how much the personal support person will be involved in meetings and decision making
- Clinicians will document involvement and current contact details of the consumer's personal support person on the FSFHG Consent for Sharing of Consumer Information form on commencement/ admission to the service.
- Individuals identified as a personal support person by the consumer may be given information about:
 - the consumer's mental health diagnosis and prognosis
 - proposed treatment plan and next steps
 - what behaviour is likely to occur and how to manage it
 - medications – benefits hoped for, importance of adherence, possible side-effects, and withdrawal effects.
 - information about local carer support groups, and how to access them

When the person experiencing mental illness does not consent to sharing information, it is important to explore this decision with the individual as it may be possible to negotiate what information may be shared and with whom [Good Practice Guidelines for Engaging with Families and Carers in Adult* Mental Health Services](#)

3.2. Did not attend procedure

- When a consumer does not attend a scheduled appointment (including group program) the staff member who was expecting to see the consumer is

responsible for making every effort to contact the consumer as soon as possible by:

- contacting the consumer by phone to identify why they did not attend and barriers preventing attendance
 - if unable to contact the consumer, contact the personal support person, family, or carer to check consumer's wellbeing
 - if there is no answer when calling the consumer or personal support person, a message is to be left on their phone via the communication method used by the treating team either SMS, durapage and or voicemail advising an appointment has been missed and the service is to be contacted
- If unable to contact the consumer or personal support person/ or concerns are raised by the personal support person, the staff member is then responsible for escalating to the treating team and will communicate actions taken to the personal support person where appropriate.

On occasions disclosure of information to personal support person requires an ability to negotiate the ethical issues around confidentiality, consent, and duty of care. If this is a concern staff will escalate the decision to the treating consultant.

3.3. Risk assessment and formulation

- If unable to contact the consumer /personal support person, the clinician who was scheduled to see the consumer will assess the consumer's risk by:
 - inclusion of the personal support person in assessment of risk where possible
 - checking PSOLIS/webPSOLIS to identify whether the consumer has had contact with another mental health service.
 - reviewing the consumer's history and recent contacts documented in the Digital Medical Record (DMR); for some consumer's disorganisation can be a significant symptom of relapse.
 - reviewing the most recent RAMP and DMR documentation regarding risk profile considering any recent changes in behaviour which may indicate a deterioration in mental state including dynamic risk factors not accounted for previously.
 - consider completing a new RAMP in the absence of the consumer to assist in identifying and communicating a deterioration, for formulation of a new risk profile and to assist in developing an escalation plan.
- Where a consumer does not attend their initial appointment with the service all known information will be reviewed including referral letter, any discussions with the consumer, any screening and history the individual may have with the service. The referrer/GP must be communicated with to discuss and identify risks/actions to be taken in conjunction with MDT discussion

3.4. Role of the treating team

- Members of the treating team will consider subsequent actions based on the needs of the individual, collateral history, assessment of current risk and prior knowledge /understanding.
- The treating team will discuss, and document agreed actions within a clinically appropriate timeframe.

3.5. Risk deemed not to be clear and/or imminent

- Where clinicians are unable to contact the consumer /personal support person and the risk to the consumer is not thought to be clear and/or imminent the clinician will discuss the consumer at the next multidisciplinary team intake meeting including risks and document actions in the DMR.
- Actions may include:
 - determining if a new appointment is required
 - organising a consumer letter advising of the missed appointment, the date and time of the new appointment and requesting they contact the service.
 - ascertaining any barriers to attending including consideration of cultural/ language barriers and the consumer's to capacity to travel and collaborating with the consumer to resolve.

3.6. Risk deemed to be clear and imminent, or staff are concerned

- Where there are concerns, or the consumer is deemed to be at clear and/or imminent risk the clinician will:
 - actively involve the consumers personal support person in decision making where possible
 - urgently consult with other treating team professionals and where necessary other agencies
 - decisions will be made in conjunction with the treating team Consultant Psychiatrist. If unavailable, the decision should be escalated to the Head of Service in business hours and on call Consultant Psychiatrist out of hours
 - consider (in collaboration with the treating team) arranging a home visit to conduct a mental health assessment or request the Assessment and Treatment Team (ATT) to conduct an assessment after business hours
 - consider involving the police or ambulance service when appropriate
 - identify the consumer as patient of concern and continue to discuss at intake and MDT meeting until acuity resolved

3.7. Documentation

- Document all attempts to contact the consumer and/or relevant others and all decisions and formulations clearly in the DMR and on PSOLIS.

- All appropriate modes of communication must be used to ensure that information and decisions are shared in a timely manner with the relevant agencies, subject to considerations of duty of care and confidentiality
- Written communication to the referrer/GP and any shared care team members clearly identifying actions and responsibilities following agreed decision making.

4. Compliance/performance monitoring

The relevant Team Leader / Programme Manager/ Nurse Director will monitor compliance with this procedure via routine clinical incident monitoring and DNA rates for outpatient clinics and group programs.

5. Related policy documents

DoH

- [Missing person](#)
- [Clinical care of people who may be suicidal](#)
- [Clinical care of people with mental health problems who may be at risk of becoming violent or aggressive](#)
- [MP 0099/18 Community Mental Health Status Assessments: Role of Mental Health Clinicians policy](#)

SMHS

- [SMHS Missing or suspected missing mental health patients](#) (SMHS:47)

FSFHG

- Clinical care of people who may be suicidal (FSFH-HW-POL-0037)
- Post Discharge follow up for mental health inpatients (FSH-MENH-POL-0019)
- Community Mental Health Status Assessment (FSFH-MENH-PRO-0006)
- Clinical risk assessment and management (FSFH-HW-POL-0017)

6. Related standards

NSQHS Standards:

- Clinical Governance
- Recognising and responding to acute deterioration

Chief Psychiatrist's Standards for Clinical Care:

- Care Planning
- Risk Assessment and Management

7. References

1. Chief Psychiatrist's Standards for Clinical Care | Chief Psychiatrist | Government of Western Australia [Internet]. Chiefpsychiatrist.wa.gov.au. 2022 [cited 19 October 2022]. Available from: <https://www.chiefpsychiatrist.wa.gov.au/standards-guidelines/chief-psychiatrists-standards-for-clinical-care/>
2. [Internet]. Chiefpsychiatrist.wa.gov.au. 2022 [cited 19 October 2022]. Available from: <https://www.chiefpsychiatrist.wa.gov.au/wp-content/uploads/2017/04/Good-Practice-Guidelines-for-engaging-with-families-and-carers-May-2016.pdf>

Authorisation

EXECUTIVE SPONSOR: Nurse Director Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2015	CN, MH	CGC MH	Policy & Legislative Compliance Committee	08/2018
2	02/2018	CNS, MH	MH SQRC		
3	07/2019	CNS, MH	MH SQRC		
4	09/2020	CNS, Clinical Improvement	Service 5 SQR Committee	FSFHG Policy Committee	09/2022
5	09/2022	CNS, Clinical Improvement	NMPGC	Service 5 SQR Committee	08/2024
6	11/2023	CNS, Clinical Improvement		Service 5 SQR Committee	07/2026